

ICD-0-3
 Carcinoma, infiltrating duct, NOS 8500/3
 Site: breast, NOS C50.9 hr 5/24/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy / History		
Dual/Synchronous Primary Noted		
Case In (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MB	Date Reviewed 5-20-11

DIAGNOSIS

(A) SENTINEL LYMPH NODE #1, RIGHT AXILLA, BIOPSY:

One benign lymph node (0/1).
 Cytokeratin stain shows no evidence of metastatic carcinoma.

(B) SENTINEL LYMPH NODE #2, RIGHT AXILLA, BIOPSY:

One benign lymph node (0/1).
 Cytokeratin stain shows no evidence of metastatic carcinoma.

(C) RIGHT BREAST, SEGMENTAL MASTECTOMY:

INVASIVE DUCTAL CARCINOMA OF BREAST, WITH SOLID PAPILLARY GROWTH PATTERN, MODIFIED BLACK'S NUCLEAR GRADE 1 (WELL DIFFERENTIATED).

INVASIVE CARCINOMA MEASURES 2.7 CM IN GREATEST DIMENSION.

Lymphatic/vascular invasion not identified.

INTRADUCTAL CARCINOMA (DCIS), MODIFIED BLACK'S NUCLEAR GRADE 1 (LOW GRADE), CRIBRIFORM AND PAPILLARY TYPES, WITHOUT NECROSIS.

INTRADUCTAL CARCINOMA (DCIS) PRESENT ADMIXED WITH AND ADJACENT TO THE INVASIVE COMPONENT, COMPRISING APPROXIMATELY 5% OF THE TUMOR.

Surgical margins are free of tumor.

INVASIVE CARCINOMA IS 1 MM TO THE CLOSEST ANTERIOR (SUPERFICIAL) MARGIN, 5 MM TO THE CLOSEST POSTERIOR (DEEP) MARGIN.

INTRADUCTAL CARCINOMA (DCIS) IS 2 MM TO THE CLOSEST ANTERIOR (SUPERFICIAL) MARGIN.

Biopsy site changes.

Focal fibrocystic change.

(D) ADDITIONAL SUPERIOR MARGIN AND LESION AT 9 O'CLOCK, EN BLOC EXCISION:

Benign breast parenchyma, no evidence of malignancy.

A small capillary hemangioma.

Fibrocystic changes including focal ductal hyperplasia with and without atypia, columnar cell alteration, apocrine metaplasia, fibroadenomatoid change, cyst formation and pseudoangiomatoid stromal fibrosis.

Microcalcifications are present in benign ducts.

(E) SUPERFICIAL INFERIOR MARGIN, EXCISION:

Benign breast parenchyma, no evidence of malignancy.

Fibrocystic changes including focal florid ductal hyperplasia without atypia, columnar cell hyperplasia, apocrine metaplasia and cyst formation.

(F) ADDITIONAL DEEP MARGIN, EXCISION:

Benign breast parenchyma, no evidence of malignancy.

(G) ADDITIONAL MEDIAL MARGIN, EXCISION:

Benign breast parenchyma, no evidence of malignancy.

(H) ADDITIONAL SUPERFICIAL MEDIAL MARGIN, EXCISION:

Benign breast parenchyma, no evidence of malignancy.

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 TCGA-GM-A2DM-01A-PR

Redacted



(I) ADDITIONAL SUPERFICIAL LATERAL MARGIN, EXCISION:

Benign breast parenchyma, no evidence of malignancy.

(J) ADDITIONAL SUPERFICIAL INFERIOR MARGIN, EXCISION:

Benign breast parenchyma with focal minimal atypical ductal hyperplasia.

Entire report and diagnosis completed by _____

COMMENT

Please see _____ for the results of prognostic markers.

GROSS DESCRIPTION

(A) RIGHT AXILLARY SENTINEL LYMPH NODE #1, BLUE, COUNT 8 - A single ovoid, pink-tan lymph node (2.0 x 1.0 x 1.0 cm) is serially sectioned and touch preps made (touch preps 1-4) and entirely submitted in cassettes A1 and A2.

*TP/DX: NO TUMOR PRESENT.

(B) RIGHT AXILLARY SENTINEL LYMPH NODE #2, BLUE, COUNT 153 - A portion of yellow fibroadipose tissue (4.0 x 2.0 x 1.0 cm) contains a single fatty, gray to white lymph node (2.8 x 1.8 x 0.8 cm). The lymph node contains a 0.4 cm white nodule. The specimen is serially sectioned, touch preps 1-4 made and a frozen section done, submitted entirely in cassettes B2 and B3.

*FS/DX: NO TUMOR PRESENT.

(C) RIGHT SEGMENTAL MASTECTOMY, SHORT DOUBLE STITCH SUPERIOR, LONG DOUBLE STITCH LATERAL, LONG SINGLE STITCH DEEP - A right segmental mastectomy specimen, oriented by the surgeon, 7.8 x 6.0 x 1.5 cm. The specimen was sectioned from superior to inferior in eight slices. Within the center of the specimen is a 2.7 x 2.5 x 1.7 cm well-circumscribed pink-tan friable mass, located at the anterior margin, 1.3 cm from the lateral, 2.2 cm from the medial margin and 2.0 cm from the deep margin.

There is a 0.5 x 0.4 x 0.4 cm nodular firm area in slice #6, possibly representing previous biopsy site, 0.3 cm from the medial margin and 1.0 cm from the superficial margin. The breast tissue surrounding the mass is composed of greater than 90% yellow adipose tissue. No additional masses are identified.

INK CODE: Blue - superior, green - inferior, yellow - anterior, black - deep, red - medial, orange - lateral.

SECTION CODE: C1, slice #4, lateral; C2, slice 4, lateral; C3, slice 4, tumor and anterior margin; C4, slice 4, deep margin; C5, slice 4, medial margin; C6, slice 5, lateral margin; C7, slice 5, tumor and anterior margin; C8, slice 5, tumor and deep margin; C9, slice 5, anterior; C10, slice 5, deep margin; C11, slice 6, tumor and medial margin; C12, slice 7, tumor and anterior margin; C13, slice 7, medial/anterior margin; C14, slice 8, inferior margin; C15, slice 1, superior margin; C16, slice 3, representative uninvolved breast tissue; C17, slice 6, 0.5 cm nodule with anterior/medial margin.

(D) EN BLOC EXCISION, ADDITIONAL SUPERIOR MARGIN AND LESION AT 9 O'CLOCK (STITCH SUPERIOR MARGIN) - A tan-yellow fibrofatty irregular soft tissue fragment (7.0 x 5.5 x 2.0 cm) with a suture at an edge of the specimen. The specimen is arbitrarily oriented in the form of a clock with suture designated as 12 o'clock. The face of the clock has a fibrotic area (3.0 x 2.0 cm). The back of the clock is predominantly fatty. The specimen is serially sectioned from 3 to 9 o'clock. The fibrotic area seen on the face of the clock extends to the 12 o'clock edge and measures 4.0 x 3.5 x 0.8 cm. The remainder of the specimen shows fatty tissue.

INK CODE: Green - 9 to 3 o'clock edge. Black - 3 to 9 o'clock edge. Yellow - face of the clock. Blue - back of the clock.

SECTION CODE: D1-D39, specimen entirely submitted from 3 to 9 o'clock.

(E) SUPERFICIAL INFERIOR MARGIN, STITCH TRUE MARGIN - A tan-yellow fatty fragment (5.0 x 4.0 x 1.7 cm) with a stitch on one side. A 1.0 cm white firm area is present abutting the inked margin with central fat necrosis and a 0.4 cm cyst.

INK CODE: Black - true margin.

SECTION CODE: Specimen is entirely submitted in E1 to E14.

(F) ADDITIONAL DEEP MARGIN, STITCH AT TRUE MARGIN - A tan-yellow specimen (3.5 x 2.4 x 1.8 cm) with a stitch on one side. The side with the stitch is inked black. The specimen is entirely submitted in F1 to F7.

(G) ADDITIONAL MEDIAL MARGIN, STITCH = TRUE MARGIN - A tan-yellow fatty specimen (4.5 x 2.5 x 1.7 cm) with stitch on one side. The side with the stitch is inked black. The specimen is entirely submitted in C1-C8.

(H) ADDITIONAL SUPERFICIAL MEDIAL MARGIN, STITCH = TRUE MARGIN - A tan-yellow fatty tissue specimen (3.5 x 2.5 x 2.0 cm) with a stitch on one side. An irregular fragment fibrotic area is present in the specimen focally abutting the inked true margin.

INK CODE: Black - true margin.

SECTION CODE: Specimen is entirely submitted in H1-H8.

(I) ADDITIONAL SUPERFICIAL LATERAL MARGIN - A tan-yellow fatty tissue specimen (2.9 x 1.5 x 0.9 cm) with suture on one side. The suture side is inked black. The specimen is entirely submitted in I1-I3.

(J) ADDITIONAL SUPERFICIAL INFERIOR MARGIN, STITCH = TRUE MARGIN - A tan-yellow fatty tissue specimen with focal fibrosis (3.0 x 3.0 x 1.0 cm) with a stitch on one side. The side with the stitch is inked black. The specimen is entirely submitted in J1-J5.

CLINICAL HISTORY

None given.

SNOMED CODES

T-04050, M-85003, M-85002

"Some tests reported here may have been developed and performance characteristics determined by specifically cleared or approved by the U.S. Food and Drug Administration."

These tests have not been

Released by:

-----END OF REPORT-----